Office of Professional Licensure Review
UTAH DEPARTMENT OF COMMERCE

2025 Periodic Review

Physical Therapy

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Executive Summary

Background

Physical therapists (PTs) are licensed healthcare professionals who examine, diagnose, and treat movement dysfunction and neuromusculoskeletal conditions caused by disease, injury, and disability. Physical therapist assistants (PTAs) are educated and licensed healthcare providers who provide care under the direction and supervision of a PT; PTs determine the plan of care while PTAs help implement treatment.

Utah requires PTs and PTAs to be licensed. Education requirements, such as graduation from an accredited education program and passing an examination, are common in states throughout the country. Although scope of practice is largely uniform, states differ in whether they allow PTs to engage in specific practices such as ordering imaging, prescribing durable medical equipment (DME), and dry needling, among others. Utah has historically enabled PTs to work to the full extent of their training and has allowed unrestricted direct access to PTs (with no physician referral) since the 1980s.

Regulatory Model Recommendation: *Continue to license PTs*

- PTs provide complex care to often physically vulnerable patients that, if performed poorly, may seriously exacerbate injuries and cause future issues
- Some PTs provide care independently and without the oversight of others

Recommended Regulatory Adjustments:

1) *Expand PTs imaging privileges*
    - PTs are educated on a variety of imaging methods beyond X-ray and MRI imaging currently allowed in statute
    - Research suggests that PTs follow proper ordering standards and do not over-utilize imaging
2) *Allow PTs to order laboratory tests*
    - PTs are increasingly being educated and trained on incorporating diagnostic laboratory tests into their practice.
    - This may lead to small efficiency gains and patient benefits, as patients would not necessarily need a referral to other healthcare providers for labs.
3) *Allow PTAs to perform low-velocity non-thrust joint mobilizations (NTJs)*
    - Nearly all PTA programs teach these techniques, including Utah programs
    - Most states besides Utah allow PTAs to perform low-velocity NTJs

Additional Recommendations:

4) *Increase the supervision ratio for supportive personnel*
    - Extending the supervision limit would increase PTs' flexibility in using PTAs and unlicensed aides while retaining a reasonable standard of care
5) *Update the definition of dry needling*
    - The current definition is unnecessarily limiting and few states that allow dry needling restrict the scope to only trigger points (as Utah does)
    - Utah has rigorous training requirements for dry needling relative to other states

Context

Consistent with its legislative mandate,[1] the Office of Professional Licensure Review (OPLR) reviewed Utah's licensing laws for physical therapists. The review evaluated how well current regulations:

1. Protect the public from present and consequential physical and financial harm
2. Balance public and practitioner access to the occupation
3. Limit the economic impact of regulation on consumers, practitioners and the state[2]

OPLR's research for this review included analysis of Utah's current laws and rules, licensing and complaint data from the Division of Professional Licensing (DOPL), licensee survey results, academic literature, health workforce data, as well as laws and policies in other states. OPLR also conducted interviews with physical therapists, business owners, employers, faculty, and other subject matter experts. See Appendix 1.1 for more information.

Background

Profession Overview

Physical therapists (PTs) are licensed healthcare professionals who examine, diagnose, and treat movement dysfunction and neuromusculoskeletal conditions caused by disease, injury, and disability.[3,4] In doing so, PTs help improve patients' physical function, manage pain and other chronic conditions, and help patients recover from and prevent further injuries. Physical therapy treatment includes the use of therapeutic exercise, manual therapies, patient education, assistive devices, and other therapeutic modalities indicated in a patient's unique PT treatment plan.

PTs work with supportive personnel. Physical therapist assistants (PTAs) are "educated and licensed clinicians who provide care under the direction and supervision of a licensed physical therapist. PTAs implement components of patient care, obtain data related to the treatment provided, and collaborate with the physical therapist to modify care as necessary."[5,6] Unlicensed PT aides have no formal training or education and provide "routine assistance", such as preparing PT equipment or providing limited elements of patient care, to PTs and PTAs under on-sight supervision.[7]

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[1] UCA 13-1b-203(2)
[2] UCA 13-1b
[3] APTA, Presentation on The Physical Therapy Profession
[4] See UCA 58-24b for the legal scope of practice for physical therapists and assistants in Utah.
[5] PTAs work under general supervision of a PT
[6] APTA, Becoming A Physical Therapist Assistant
[7] Routine assistance may include performing operational tasks necessary to provide physical therapy or providing direct patient care for tasks and exercises that the patient or their family could be reasonably expected to learn. See UCA 58-24b and UAR R156-24b

PTs & PTAs treat a wide range of neuromusculoskeletal conditions and patient ages in a variety of settings. Their work ranges from the rehabilitation of simple sprains to the management of complex disorders, such as Parkinson's disease or multiple sclerosis. PTs most frequently work in outpatient physical therapy clinics, followed by hospital and acute care settings, with a small number in home health care and skilled nursing facilities.[8]

Evolution of Physical Therapy

Physical therapy has evolved dramatically over the last century. It first emerged as a distinct profession in the early twentieth century, with bachelor's-trained PTs working under physicians in the care and rehabilitation of disabled veterans and those impacted by the polio epidemic. Advancements in medicine, surgery, and science prompted increased demand for rehabilitative healthcare, prompting the evolution of physical therapy. In 1972, the U.S. Army allowed PTs to adopt a primary-care provider role for patients with neuromusculoskeletal disorders, and in the decades following PTs gained similar autonomy in civilian sectors.[9] In the last 25 years, minimum PT education has increased significantly alongside scope. A master's degree (in 2002) and now a doctoral degree (in 2016) have become the minimum entry requirement for PTs. Today, each state allows individuals to see PTs without physician referral, highlighting both their expanded scope of treatment and role in the healthcare system.

Profession in Utah

Utah has 3,193 licensed PTs and 1,114 licensed PTAs.[10] To become licensed as a PT in the state, an individual must graduate from a physical therapy program accredited by the Commission on Accreditation of Physical Therapy Education (CAPTE) and pass the National Physical Therapy Examination (NPTE-PT).[11] CAPTE only accredits Doctor of Physical Therapy (DPT) programs.

A PTA seeking licensure in Utah must graduate from a physical therapist assistant education program accredited by CAPTE (at least Associate's level) and pass the NPTE-PTA.[12] Both PT and PTA licensees must renew their license every two years; to renew, PTs must complete forty continuing education units and PTAs must complete twenty. [13]

[8] Results informed by the DOPL renewal survey, DWS Physical Therapist Overview, and APTA (2023) A Physical Therapy Profile:Demographics of the Profession. The same sources indicate that PTAs tend to work in outpatient settings as well as in skilled nursing facilities and home health. PTAs do not work as frequently in hospital and acute settings.
[9] Clark et. al. (2021) A call to action: direct access to physical therapy is highly successful in the US military. When will professional bodies, legislatures, and payors provide the same advantages to all US civilian physical therapists?
[10] DOPL Active Licensees Report, accessed 8/25/2025.
[11] The NPTE-PT is administered by the Federation of State Boards of Physical Therapy (FSBTP).
[12] CAPTE accredits both associate's and bachelor's PTA programs
[13] UCA 58-24b

Approaches in Other Jurisdictions

Every state in the U.S. licenses both PTs and PTAs. The requirements for licensure are largely uniform across the nation. At a minimum, every state requires applicants to graduate from a CAPTE accredited program and pass the applicable NPTE exam.[14] In addition to this, twenty-nine states require PT applicants to pass a jurisprudence assessment and five states require professional liability insurance; Utah requires neither.[15]

Although requirements are fairly standardized, the scope of practice of these licenses varies slightly across states. Utah has historically enabled PTs to work to the full extent of their training; the state is one of seven which explicitly allow PTs to order imaging and has allowed unrestricted direct access (with no physician referral) to PTs since the 1980s.[16,17]

Regulatory Model Assessment & Recommendation

The Framework

In an effort to standardize how appropriate regulatory models are determined for each profession (e.g. license, registry, no regulation, etc.), OPLR developed a framework which incorporates its statutory review criteria.[18] Appropriate models are determined principally by an evaluation of the potential for harm and related factors that may aggravate or mitigate the potential for harm. These factors include the availability of consumer choice, vulnerability of patients, and independence of practice. See Appendix 2.1 for potential regulatory models and the factors in OPLR's framework.

Recommendation

OPLR determined that licensing is an appropriate form of regulation for physical therapy. Significant considerations for this recommendation include: the potential for harm inherent in the practice of physical therapy, the downstream consequences of untrained care, and the level of independence within physical therapy which is not offset by other factors such as strong alternative forms of oversight or patient choice based on information.[19]

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[14] FSBPT Jurisdiction Licensure Reference Guide: Requirements for Licensure by Examination
[15] Ibid.
[16] OPLR Policy Scan. 7 other states also consider ordering imaging in the scope of PT but have not codified it in law. All other states are either "silent" or explicitly restrict image ordering.
[17] Ibid. 32 states do not have unrestricted direct access; they either have direct access with provisions or have limited direct access.
[18] Among other criteria, OPLR is required to evaluate "whether the regulation of the occupation is necessary to address a present, recognizable, and significant harm to the health, safety, or financial welfare of the public" and consider "potentially less burdensome alternatives to the… existing regulation". UCA 13-1b-302
[19] This is not a comprehensive list of all factors. Please see Appendix 2.2 for more information on OPLR's analysis of the potential for harm related to physical therapy.

Potential for harm

The practice of physical therapy often includes manipulation of limbs and joints, known as manual therapy, that may require strong physical force and often requires frequent physical touch of the patient. Poor or untrained performance of this scope may exacerbate a patient's current injury or disability, cause new musculoskeletal injuries, or potentially increase the rate of physical decline or immobility in a disabled patient. The potential for causing severe harm is lower in PT compared to other healthcare practitioners like physicians, nurses, and respiratory therapists, where negligent practice may result in death. However the consequences of poor PT can be severe. PTs frequently treat vulnerable, acute patients for whom errors that may not permanently harm the average individual (i.e. applying too much force, failing to prevent falls) may cause severe damage.

The potential for harm inherent in physical therapy is exacerbated by the downstream implications of untrained care. Failure to properly diagnose and/or treat neuromusculoskeletal disorders may necessitate further, potentially invasive, corrective medical interventions from other healthcare providers. Those interventions may come with high costs and additional risk. If a patient does not seek those treatments, their quality of life, ability to move and live independently, and their contributions to the community may suffer.[20]

Related harm factors

Physical therapists operate with a high level of independence and varying levels of oversight. Some PTs in Utah work under sophisticated employer oversight and are integrated into interdisciplinary teams, such as in hospital systems and acute settings. However, approximately one-third of Utah PTs work in settings which may not have high levels of oversight, such as private outpatient clinics and home health.[21] There is often a level of consumer oversight in these settings, as consumers may utilize available information, like online reviews, to make informed decisions on the clinic or home health agency they use. Available information does mitigate harm to an extent.

However, relying upon consumer oversight alone is insufficient to uphold safety within physical therapy. Information on individual PTs in employed settings may be nonexistent, and patients may be assigned a PT with no input. Additionally, there is significant information asymmetry between a PT and a patient about the efficacy of the care a patient is receiving. It may take

[20] Competent PT care is associated with immediate and downstream individual and societal benefits. Untrained PT care would likely result in a loss of these benefits. See Appendix 2.3 for more information on the association between competent PT and greater mobility, lowered healthcare costs, fewer physician visits, fewer surgeries, and less prescription opioids, among other benefits.

[21] The DOPL renewal survey found that 35% of PTs currently practicing work in private outpatient or skilled nursing home settings. The survey did not account for those working in home health, but that likely accounts for another 5-10% of PTs.

months or years for a patient to understand if they were harmed or received sub-standard care.[22] Other avenues for redress (e.g., litigation) are costly and possibly difficult to pursue.[23]

Therefore, OPLR concludes that physical therapy should continue to have a defined and protected scope of practice that only those who can prove their competency may engage in. Without state oversight, there is no adequate mechanism to stop harmful practitioners from continuing to practice. A regulatory model such as mandatory certification would sufficiently protect the scope of practice and set a bar for competence, but it would not establish the level of oversight consistent with a higher-harm profession. Ultimately, licensing is the only regulatory model that ensures these protections are in place.

Regulatory Model Adjustments & Recommendations

Framework

After determining an appropriate regulatory model, OPLR's framework also evaluates whether adjustments should be made within a recommended model to address any material and existing safety and access issues affecting the Utah public and practitioners. Regulatory model adjustments may include changing entry qualifications, the scope of practice, unprofessional or unlawful conduct, and/or supervision and independence provisions (see Appendix 3.1).

Safety Issues

OPLR found no evidence that licensed physical therapy practitioners in Utah are causing widespread, serious harm to the public based on the information available. OPLR analyzed the Department of Professional Licensing's (DOPL) complaint data and found that PTs and PTAs have among the lowest rates of substantiated complaints out of all professions reviewed in 2025.[24] A review of complaint case notes illustrated that, even among these substantiated complaints, very few involved any reported client harm.[25] Furthermore, no individual interviewed as part of this review indicated that they were aware of physical therapy licensees causing harm in any substantial or systematic way.

Access Issues

Quantitative and qualitative data on the PT workforce suggests there are some access issues, but they are not severe.

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[22] This is especially relevant in physical therapy, where treatment is often physically exhausting, difficult, and may cause some pain. Red flags may be ignored as patients assume their pain is part of the normal process rather than a consequence of poor care.
[23] Criminal law may hold practitioners accountable for severe damage if reported, but it would not restrict them from returning to practice.
[24] DOPL Complaint Analysis. Between 2017-2022, PTs had a rate of 0.67 substantiated complaints per 100 licensees, while PTAs had a rate of 0.61 substantiated complaints per 100 licensees. See Appendix 3.2 for more information
[25] DOPL Complaint Case Note Analysis. Client harm was found in only two substantiated complaints- one was a PTA who caused minor burns with a heating pack, the other was a case of sexual assault. See Appendix 3.2

The U.S. Health Resources and Services Administration's (HRSA) state-level modelling and forecasting of physical therapy supply, demand, and percent adequacy[26] suggests Utah may be experiencing a workforce shortage of PTs that could worsen in the future. The percent of demand met by supply is projected to decline from 87% in 2025 to 71% in 2037, placing Utah as the fourth worst state for PT adequacy in the nation.[27] However, HRSA modelling intentionally does not account for the recent creation or expansion of new education programs.[28] The University of Utah's recently expanded DPT program increased the educational capacity of the state by up to 65%.[29] Additionally, HRSA supply estimates are somewhat contradicted by The Utah Department of Workforce Services (DWS) employment data and DOPL licensing data. DWS estimates that employment in PT will grow by 5% *annually*, much faster than the average profession in Utah,[30] and DOPL licensing data shows a more robust increase in the number of PTs than was projected by HRSA.[31]

Qualitative data on workforce mirrors the nuanced and somewhat contradictory findings of the quantitative forecasts. Physical therapy business owners and other employers expressed some difficulty in hiring, especially in areas such as home health and skilled nursing facilities. Many reported reasonable wait-times for patients of a few days, however certain specialties- such as pelvic health- could garner wait-times of up to a few weeks. Anecdotally, stakeholders expressed concern about rural areas, which tend to have fewer healthcare providers, but survey data indicates that 17% of PTs work in rural counties despite only 10% of Utah's population living in these counties.[32,33]

The barriers to entry for individuals seeking to become physical therapists pose a more significant issue, particularly due to earnings relative to education costs. The increase in program length and cost of physical therapy education has outpaced the growth in wages. Between 2013-2021, PT salaries in the mountain west grew ~0.75% on an annualized basis while program costs grew ~2.5%.[34] Furthermore, while PTs make above the median salary in

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[26] Percent adequacy is a measure of how well forecasted supply meets forecasted demand
[27] HRSA Workforce Projections, accessed 3/12/2025.
[28] HRSA supply modeling is robust, incorporating data on licensee counts and demographic data from the APTA to identify and account for unique trends in each state. The supply forecast takes the status quo and extends it into the future to estimate the number of practitioners should no external changes occur which may inflate/deflate supply. For more information on HRSA's methodology, see: Zarek et. al. (2025) Current and Projected Future Supply and Demand for Physical Therapists From 2022 to 2037: A New Approach Using Microsimulation
[29] The University of Utah and Rocky Mountain University, the only schools offering DPT programs, had a combined capacity of up to 150 students. With this new expansion, the University of Utah has added up to 97 seats to their cohort of 50, marking a dramatic increase. 22 of these new seats will be in St. George, thereby enabling students in Southern Utah to obtain their DPT. This will likely improve access to PT in Southern Utah.
[30] DWS Physical Therapist Overview, accessed 2025
[31] HRSA projected a 1% increase in PT FTEs between 2022-2024, whereas the number of licensees grew by 11% in that same period. HRSA utilizes FTEs, not the total number of licensees, but assuming a steady rate of total FTEs in Utah (e.g., total FTEs are 80% of the number of licensees) the growth rate stays the same. See Appendix 3.3 for more information on license growth.
[32] DOPL Renewal Survey. Around 83% of PTs indicated that their primary practice location was within a county categorized as urban. County categorizations were taken from Utah's Public Health Indicator Based Information System (IBIS).
[33] HRSA (2025) Overview of the State- Utah
[34] OPLR used salary data from the Mountain West as reported in APTA (2022) A Physical Therapy Profile: Wages Earned in the Profession, 2021-2022 and used CAPTE's Aggregate Program Data Archive for program cost estimates.

Utah, they do graduate with substantial debt: $60k-80k on average and greater for more recent graduates.[35,36] These factors could dissuade individuals from entering the profession.

Insurance issues can constrain PT earnings and practice which in turn affects access. CMS reduced the amount Medicare pays for each PT service every year between 2020 and 2025, which dampens PT income, and decreased reimbursement rates significantly for PTAs in CY 2020 [37,38] Many Utah PTs, especially those not affiliated with large hospital systems, face significant frustration with insurance reimbursement. They report difficulties joining insurance company provider networks, and the administrative tasks required by insurance companies take time away from direct patient care.[39]

OPLR did not find evidence that Utah is facing a shortage of PTAs. HRSA projects PTA percent adequacy in Utah will grow to 154% by 2037.[40] Additionally, PTA barriers to entry are far lower due to shorter programs with higher and more immediate return on earnings from the cost of PTA education, particularly from a Utah public higher education institution. A PTA license requires an associate's degree, although some receive bachelor's degrees, with costs ranging between $13k-$57k in Utah, while PTAs in the state earn between $41k-$61k.[41] In Utah, most PTAs earn an associate's degree, with debt between $1-$20k on average.[42]

The discrepancy in education requirements and return on investment between PTA and PT programs may have implications for PT access. OPLR found that only 3% of PTAs licensed in Utah later became licensed as PTs in Utah.[43] Stakeholder interviews suggest program return on investment and work-life balance are key considerations for students choosing between PT and PTA programs. Additionally, transitioning from an associate's level PTA to a doctoral level PT requires significant investment, as individuals must return to school to graduate with a bachelor's before starting a DPT. Utah is developing programs to help address this gap in the career ladder, but it is too early to determine if this will have a meaningful impact on the supply of PTs in Utah.[44]

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[35] DOPL Renewal Survey. This figure takes into account all PTs who responded to the survey, regardless of when they graduated. The average debt increases to $80k-$100k when considering only those that completed their program in the last fifteen years.
[36] In Utah, PTs make a median annual salary between $66k-$98k, while DPT programs cost between $82k-160k. OPLR used DWS for salary estimates and program websites for cost.
[37] CMS CY 2025 Therapy Services Updates. The CY 2020 reimbursement cut specifically applies to services in which a PTA performs more than 10% of the total services.
[38] APTA (2025) Takeaways From the Proposed 2026 Medicare Physician Fee Schedule
[39] OPLR interview series
[40] HRSA Workforce Projections, accessed 3/12/2025.
[41] Income estimate from DWS. Program costs are from program websites: Utah Tech and SLCC each cost around $13k, while Provo College (private program) reported their 2023 program cost at $57k.
[42] DOPL Renewal Survey. Additionally, 40% of PTAs graduate with no debt. Unlike with PTs, the average debt at graduation remains $1-$20k when only considering the cohort that graduated in the last fifteen years.
[43] DOPL Licensee Data
[44] OPLR Stakeholder Interviews.

Discussion

Given evidence that current licensing requirements and enforcement mechanisms appear to mitigate the potential for harm, adjustments to up-regulate the profession of physical therapy would be unnecessary and burdensome.

Access, both to the profession and to physical therapy services, is a concern, but it is not one that can be solved primarily through licensure model adjustments. Lowering the requirements for licensure as a physical therapist may help decrease costs and increase the supply of physical therapists, but it would also require significant investment by the state and disruption to the national status quo.[45]

Ultimately, OPLR does not recommend making any large adjustments to the current licensing model. However, OPLR proposes making small, targeted adjustments to expand the scope of practice for physical therapists to address some inefficiencies in the healthcare system and enable PTs to practice to the top of their expertise. The following should be added or clarified:

Recommendation 1: Expand physical therapists' imaging privileges

Currently, Utah's Radiology Practice Act explicitly allows PTs to order only X-rays and MRIs. OPLR recommends removing language that specifies what types of diagnostic imaging may be ordered, thereby opening up to any type of imaging to PTs (i.e., CT scans, ultrasounds, bone scans).[46]

OPLR contends that PTs should be able to order (but not interpret) any type of diagnostic imaging when acting within the scope of their license, education, and expertise. Today, PTs are educated on proper utilization of a variety of imaging methods, and PT education on this topic continues to expand.[47] Broadening the definition would allow PTs currently trained on non-MRI/X-ray imaging to practice to the top of their expertise, and it leaves space for the practice to evolve as imaging technology and physical therapy education evolve.

Expanding imaging privileges is not likely to negatively impact the financial or physical wellbeing of the public. Although excessive imaging could result in increased healthcare costs and risk to a patient's health, research suggests that PTs do not over-utilize imaging and that they follow proper ordering standards.[48,49,50] Additionally, Utah statute restricts practitioners from performing

[45] There are no alternatives to CAPTE accredited DPT programs or to the FSBPT NPTE exam, which requires individuals to graduate from a CAPTE accredited program to sit for the exam.
[46] Arizona recently passed a bill to do this: AZ HB2583
[47] PTs graduating now must be educated in imaging. In 2016, the Commission on Accreditation for Physical Therapy Education (CAPTE) mandated imaging curricula in entry-level physical therapy education as a requirement for accreditation, and entry-level diagnostic imaging competence is evaluated within the National Physical Therapy Examination (NPTE).
[48] Keil et. al. (2019) Ordering of Diagnostic Imaging by Physical Therapists: A 5-Year Retrospective Practice Analysis
[49] Mabry et. al. (2020) Safety Events and Privilege Utilization Rates in Advanced Practice Physical Therapy Compared to Traditional Primary Care: An Observational Study
[50] Keil et. al. (2025) Ordering of diagnostic imaging by physical therapists: a multi-center analysis of successful implementation

any act that is outside of their training or expertise.[51] Conversely, the current restriction on imaging may increase cost, although this is not well documented. Requiring PTs to refer a patient when they themselves are capable of ordering imaging may increase cost and decrease efficiency; a patient may have to wait longer and/or pay an additional co-pay.[52]

Recommendation 2: Allow PTs to order laboratory tests

Utah law does not allow PTs to order any laboratory tests. OPLR recommends expanding the scope of practice of physical therapy to include ordering and reviewing PT-related laboratory tests (i.e., INR and blood tests evaluating levels of hemoglobin and hematocrit, white blood cell, arterial blood gas, among others), if the following criteria are met:

1. The PT has received education and/or training on best ordering practices for laboratory tests and understands the implications of possible results,
2. The laboratory test is relevant to the practice of physical therapy and treatment of musculoskeletal disorders,
3. The PT has entered a physical therapist-patient relationship with the patient for whom there is a clinical need for the order, and
4. The PT has a duty to refer to an appropriate health care practitioner if the laboratory test results cause the PT to believe that any symptoms or conditions are present that may require services beyond the PT's scope of practice.

Ordering laboratory tests does not include taking or analyzing samples. Instead, PTs would evaluate whether the lab reported values fall within a "normal" range for PT-related conditions and use those findings to adjust the plan of care or refer to another practitioner if necessary.[53] Expanding the scope of PT may lead to small efficiency gains and patient benefits, as patients would not necessarily need additional referral to other healthcare providers just to get a test ordered. Patients using PTs as their primary point of contact in the healthcare system may also benefit from faster referral to physicians from PTs as needed (e.g., if lab results indicate an underlying condition that requires physician care).

OPLR found that PTs are increasingly being educated and trained on incorporating diagnostic laboratory tests into their practice. Although CAPTE does not require DPT curriculum to include utilizing laboratory results specifically, patient screening and examination is a core competency for curriculum in accredited programs,[54] which often includes the use of laboratory tests. Additionally, both DPT programs in Utah include education on the interpretation and implications of laboratory tests across multiple core classes.[55]

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[51] UCA 58-1-501(2)(ix)
[52] A small academic study found that allowing PTs to order X-rays created savings for individuals and the healthcare system. See: Peterson et. al. (2021) Extended roles in primary care when physiotherapist-initiated referral to X-ray can save time and reduce costs
[53] PTs may not make a formal "medical diagnosis"
[54] CAPTE (2024) Standards and Required Elements for Accreditation of Physical Therapist Education Programs
[55] Based on correspondence with instructors at both DPT programs in the state. One faculty member stated that students are taught and tested on interpreting lab values within cardiopulmonary, neuromuscular management,

Furthermore, OPLR believes that expanding PT scope in this manner poses little risk to patient health. Having blood drawn by a phlebotomist is unlikely to result in major complications. Additionally, the stipulations above set guardrails as to when, why, and for whom PTs may order laboratory tests, decreasing the likelihood of neglecting proper diagnostic protocols.[56] Unfortunately, OPLR found no studies with estimates regarding risk. This is likely because Utah would be the first state to explicitly allow PTs to order any diagnostic laboratory testing.[57]

Recommendation 3: Allow PTAs to perform non-thrust joint mobilization

OPLR recommends removing the total prohibition currently restricting PTAs from practicing any type of joint mobilization and allow them to perform low velocity, non-thrust joint mobilizations (NTJ), as long as the supervising physical therapist ensures the PTA has the training and education to safely and competently perform the act.[58]

Joint mobilizations are very common manual therapy techniques to relieve pain and increase mobility. Non-thrust joint mobilizations are "low-velocity and often repeated joint movements of varying amplitude"[59], which may include applying gentle pressure, stretching, or pulling, all within the joint's natural range of motion. NTJ can be differentiated from thrust joint mobilizations (TJM), "high-velocity low-amplitude procedures characterized by a single, short duration thrust directed at a target joint that often results in an audible sound, or cavitation".[60] TJM includes applying a much higher force applied at the end range of joint movement.

Few states besides Utah explicitly restrict PTAs from performing NTJs.[61] Nearly all PTA programs teach NTJs, including those currently in Utah.[62,63] There is high demand among employers for PTA competence in NTJs,[64,65] and expanding their scope may help improve patient access to care. The ability to delegate common manual therapy techniques like NTJ may allow PTs to spend more time on higher complexity activities and see more patients.

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pharmacology, and complex patient management classes/curriculum. Additionally, the NPTE includes questions on ABG and other lab value interpretation.

[56] There are also existing resources for PTs to refer to on this topic. The APTA has published a resource, updated regularly, on Laboratory Values Interpretation which contains information on lab tests relevant to PT, how patient characteristics influence results, and the normal range of results for all included lab tests. See: APTA (2023) Laboratory Values Interpretation Resource

[57] Maine allows PTs to perform a finger stick blood test to assess blood clotting and the military allows PTs to order diagnostic laboratory tests.

[58] UCA 58-24b-402 states that PTAs may only do "simple joint distraction techniques or stretching; or a stretch or mobilization that can be given as part of a home exercise program", activities which an unlicensed aide may also perform.

[59] Xia et. al. (2016) Similar Effects of Thrust and Non-Thrust Spinal Manipulation Found in Adults With Subacute and Chronic Low Back Pain – A Controlled Trial with Adaptive Allocation

[60] Ibid.

[61] OPLR Policy Scan. Only 5 states and D.C. fully prohibit PTAs. The vast majority of states stay "silent" on the issue, allowing the PT to delegate to a PTA manual therapy techniques that the PT determines are in the scope of expertise of the PTA.

[62] A 2016 survey of accredited PTA programs found that "Grades I and II mobilizations are didactically taught by 98% and 99% of the programs respectively with grades III and IV included by over 80% of the programs". See: Gorman (2017) Teaching Joint Mobilizations in Physical Therapist Assistant Programs: Implications for Instruction and Policy

[63] Correspondence with program administrators at Salt Lake Community College and Utah Tech University

[64] Gorman (2017)

[65] OPLR Interview Series

Unfortunately, very little data exists on the safety of performing NTJs, as much of the literature focuses on TJMs,[66] which carry more inherent risk due to the force and movement involved. A few small trials on NTJs and TJMs found no adverse events, but these studies included only PTs.[67] Despite a lack of quality data, OPLR does not believe expanding the scope of PTAs in this way poses a significant risk to the public. PTA programs teach these techniques, few states explicitly prohibit PTAs from performing NTJs, and ultimately, the supervising PT is responsible for delegating to a PTA only what the PTA can safely perform.

Additional Recommendations

OPLR also recommends removing unnecessary limitations to the practice of physical therapy posed by supervisory constraints.

Recommendation 4: Increase the supervision ratio for supportive personnel [68]

PTs in Utah may only supervise up to three full-time supportive personnel at a time, which includes any ratio of licensed PTAs and unlicensed PT aides. OPLR recommends extending this limit to five supportive personnel, of which up to three may be PTAs.

PTAs play a pivotal extender role in physical therapy by allowing the supervising therapist to work on higher complexity tasks[69] while PTAs carry out less complicated aspects of a patient's care plan. Utah's limit on supportive personnel is restrictive, as utilizing two aides would limit a physical therapist to only one physical therapist assistant. Using fewer or no aides reduces the functionality of a PTA, who may have to assume responsibilities of an unlicensed aide. This may impact access to care. Extending the supervision limit would increase PTs' flexibility in using PTAs and unlicensed aides while retaining a reasonable standard of care.

This is consistent with other states; 24 states have a higher supervision cap than Utah's current cap of three.[70]

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[66] Quality reporting on adverse events for TJMs is poor as well. See: Funabashi et. al. (2022) Definition and classification for adverse events following spinal and peripheral joint manipulation and mobilization: A scoping review

[67] See: Creighton et. al. (2014), Masaracchio et. al. (2013), Coulter et. al. (2018)

[68] A few health care leaders have expressed concern that extending the supervision ratio may introduce inefficiencies in care in certain circumstances. However, this recommendation does not require any employer or facility to use higher ratios. Rather, it allows employers and facilities flexibility to innovate and use higher ratios when they deem it effective or necessary to provide timely care. Additionally, OPLR did not find any evidence that this may endanger patient safety. PTAs have few complaints, and interviews suggested the concern regarded efficiency rather than safety.

[69] Higher complexity tasks may include diagnosis, treatment planning, or implementing manual therapy techniques like dry needling or thrust joint mobilization.

[70] OPLR Policy Scan

Recommendation 5: Update the definition of dry needling

Utah first granted PTs the authority to perform dry needling in 2014, becoming one of the first states to explicitly allow the practice in statute.[71] Utah's 2014 bill defined the practice of dry needling very narrowly as the stimulation of trigger points only. OPLR recommends updating the definition from "trigger point dry needling" to "dry needling" to better align with other states and the reality of the practice.[72,73]

Dry needling targets trigger points, "hyperirritable spots, usually within a taut band of skeletal muscle", which are commonly referred to as painful "knots" in muscle. Treating these trigger points often includes inserting needles into the point itself, but it may also include inserting needles into proximal and distal points (points nearer and farther from the actual trigger point).This can be necessary for safety reasons, such as when touching the trigger point causes too much pain or there is a wound in that spot.[74] Utah's definition prohibits PTs from targeting these proximal and distal points, thereby restricting PTs from fully practicing dry needling.

Very few states that explicitly allow PTs to dry needle limit the scope to only trigger points.[75] Utah also requires the second highest number of training hours for PTs to legally perform dry needling.[76] Re-defining dry needling more broadly would align state law with Utah's already rigorous training requirements and the current full scope of this practice.

Other Considerations

Along with the stated recommendations, OPLR considered granting limited prescriptive authority for physical therapists to prescribe topical analgesics and Nonsteroidal Anti-Inflammatory Drugs (NSAIDs). Topical analgesics and NSAIDs are useful in the context of physical therapy as they help decrease pain and inflammation. Both medications are available over-the-counter; however, prescribing privileges would allow PTs to order medications with higher dosages (i.e. 800mg ibuprofen or 5% lidocaine cream). OPLR determined that, although rare, topical analgesics and NSAIDs can cause severe harm,[77] and physical therapy education has not yet incorporated enough training or education on the prescription of drugs.[78] Although PTs in the

[71] HB0367
[72] UCA 58-24b-102
[73] Specifically, OPLR recommends changing "Trigger Point Dry Needling" to "Dry Needling" and adopting the FSBPT's definition, which is "a skilled intervention using thin, filiform needles to penetrate the skin and/or underlying tissues to affect functional change in anatomical structures and systems for the evaluation, management, and prevention of neuromusculoskeletal conditions, pain, movement impairments, and disability."
[74] Dunning et. al. (2014) Dry needling: a literature review with implications for clinical practice guidelines
[75] OPLR Policy Scan and APTA (2024) State Laws and Regulations Governing Dry Needling Performed by PTs
[76] Ibid.
[77] NSAIDs have been associated with an increased risk of heart failure, stroke, gastrointestinal bleeding, and kidney disorders. Additionally, lidocaine at high doses can cause central nervous system disorders/disturbances, although this is rare given the low dosage and absorption via topical application.
[78] They do receive pharmaceutical training, but it doesn't prioritize proper prescription protocol..

military have been granted limited prescriptive authority, they must go through additional training,[79] and no U.S. state has granted PTs this authority.[80]

Therefore, OPLR recommends maintaining awareness of physical therapy program curriculum regarding these drugs. This scope of practice could be revisited in the future once PT education and/or training begins to teach and test competency on this topic.

Rule Review

In accordance with Utah Code 13-1b-203(5), OPLR conducted an in-depth review of DOPL's physical therapy practitioner rules, found in R156-24b.

The rule review covered potential rule changes needed to:

1. address specific rules that may be overly burdensome (e.g., for individuals seeking to practice a profession or given the potential risk to public safety from a profession, etc) or insufficient (e.g., to ensure safe practice);
2. address rules misaligned with statutory language, clarify language, and correct references to statute or other rules; or
3. support OPLR's recommendations

OPLR's review of R156-24b found:

1. There are multiple rules that may be overly burdensome for physical therapy practitioners:
    a. DOPL recognizes the Commission on Accreditation of Physical Therapy Education (CAPTE) as the only accreditor for education programs. This is appropriate as CAPTE is the only active PT accreditor, but OPLR recommends allowing for the possibility of accepting other accreditors should they be created to avoid a monopoly by statute. OPLR is pursuing this recommendation globally for all occupations in this review.
    b. Utah has very high continuing education (CE) requirements for PTs compared to other states. PTs must complete forty CEs per biennial renewal cycle. Only Utah and six other states require that many.[81] OPLR recommends reducing the number from forty to thirty, as thirty is the median across all states.[82]
    c. Requiring two CEs on ethics and law every renewal may be unnecessary. OPLR recommends reducing this requirement to one CE in ethics or law.
2. Nine incorrect references to statute. These are outlined in Appendix 4.1.
3. The training or education necessary for PTs to order laboratory testing should be specified in UAR R156-24b.
4. The recommendation to increase the supervision ratio of supportive personnel must be changed in UAR R156-24b-503.

[79] Benson et. al. (2014) The role of Army physical therapists as nonphysician health care providers who prescribe certain medications: observations and experiences
[80] OPLR Policy Scan
[81] OPLR Policy Scan
[82] Ibid.

Appendix

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1. Context

1.1 General Research Methodology

OPLR's methodology combines qualitative and quantitative methods with robust stakeholder engagement. Methods include:

- Analyzing data from workforce surveys administered by the Department of Professional Licensing (DOPL) as part of licensure renewal
- Conducting quantitative analysis of DOPL licensee and complaint data and publicly available data from other state and federal government entities (e.g., DWS, HRSA, NPDB)
- Reviewing academic literature and reports on a profession's practice, efficacy and safety
- Scanning education and credentialing requirements, programs and content
- Reviewing state occupational regulation policies across the U.S.
- Engaging with a wide range of stakeholders, including: Utah and other state governments and agencies, industry organizations, researchers, practitioners, and business owners and employers within a variety of settings (e.g., acute inpatient hospital, private outpatient, hospital system outpatient, home health, and skilled nursing facilities)

1.2 DOPL Physical Therapy Renewal Survey

A. Survey overview

OPLR utilized a DOPL survey available to PTs and PTAs during their 2025 renewal period for information on the workforce in Utah. This survey is administered by DOPL for use by the Health Workforce Information Center (HWIC) to inform legislators and the public about workforce trends and projections. For more information regarding the information collected, the survey instrument can be found **here**.

A. Survey Limitations

The survey was available to all physical therapy licensees during the license renewal process so results were not affected by sampling bias. Additionally, the response rate was high, around 70% for PTs and 60% for PTAs. However, results still may be affected by non-response bias (e.g., if those who chose to respond to the survey shared characteristics not representative of the true population). This may be especially true for specific survey items, like primary practice location, for which only 63% of PT survey respondents gave a legitimate response. As a result, the overall population represented for that item is roughly 45% and less likely to represent the population.

Other possible limitations include measurement error (which occurs when questions do not accurately measure the variable interest due to errors in question design) and recall bias (where

respondents misremember and inaccurately answer questions). For example, recall bias may impact the estimates of hours worked per week or debt at graduation. All of these potential errors may cause some variability or systematic bias.

OPLR uses this to provide background understanding of a profession, outline patterns, and identify general trends rather than to provide exact estimates. Therefore, the limitations articulated above should not unduly impact OPLR's findings or recommendations.

1.3 Physical Therapy Policy Scan

To better understand the regulatory environment for physical therapy, OPLR conducted a review of state occupational regulation in the U.S., which was heavily informed by The Federation of State Boards of Physical Therapy (FSBPT) and made use of the deep research function of Gemini (Google's AI tool). OPLR then validated the Gemini information by the methods mentioned below. The validated sources were then used to map the national policy landscape, find patterns in regulation, make cross-state comparisons, and discover outliers. OPLR also used the data to help inform recommendations.

OPLR used the FSBPT's licensing reference guide to understand jurisdictions' general approach to licensing.[83] This resource is validated quarterly with each jurisdiction and contains information on each state's approach to initial licensing, license renewal, and license management for physical therapy practitioners.[84]

OPLR used the deep research function of Gemini for two primary purposes: 1) to find high quality academic sources and 2) determine how jurisdictions differ on their approach to very specific policies. Gemini was used to find sources related to the benefits of physical therapy, safety of joint mobilizations, and adverse outcomes in physical therapy, among others, and OPLR used the following basic prompt:

"*Please give me academic sources that address the following topic: [topic]. [Additional item to prioritize in research]. Note sources that discuss competencies necessary to safely practice. Please use only peer-reviewed, academic sources accessed through the following databases: Scopus, Web of Science, PubMed, ERIC, Google Scholar, ScienceDirect, JSTOR, EBSCO, and Cochrane Library. Provide the sources in this chart format: Article Title, Journal or entity, Year, Key Findings, citation, Full URL Link*."

OPLR verified all academic sources by reading each source to ensure accuracy and conducted literature reviews without using Gemini when the data provided was insufficient.

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[83] FSBPT, Licensure Reference Guide

[84] OPLR utilized the following reports: Initial Licensure Requirements for PTs and PTAs, Renewal Requirements, Number of CEUs/CCUs Required, Supervision Levels by Setting, Supervision Ratios, Role of Unlicensed Personnel, Direct Access, Referral Requirements, and Jurisdiction Language Regarding Physical Therapists and Imaging. All reports can be accessed on the following pages: Licensure Reference Guide and Diagnostic and Procedural Imaging in PT Practice

OPLR also used Gemini to research how jurisdictions differ regarding specific policies, such as allowing PTAs to perform joint mobilizations, PT scope of practice as it relates to imaging privileges, authority to order laboratory tests, prescription of DME, and prescription of NSAIDs and topical analgesics, and definitions of and requirements for dry needling by PTs. The following is an example of a prompt used to research dry needling:

"*Please answer the following questions. For all U.S. states that allow physical therapists to dry needle, how do they define dry needling? What is the scope of what a physical therapist is allowed to do? How many hours of extra education are required to be able to perform dry needling?* ***Use only state websites, state administrative code, state administrative rules, and national industry organization websites to research this.*** *Do not use Reddit, blogs, or other non-verifiable sources. Please give me answers for all 50 states and D.C.,, and read each state/territories statute AND administrative rule governing physical therapy licensing. Provide answers in a chart format, where there is a column for state, the definition/scope, hours of education required, and the full URL link.* "

OPLR then verified Gemini's results across a random selection of states to ensure general reliability for research fundamental to any recommendations.

This review does contain limitations related to normal human error. It is possible that there is slight misreporting of some data due to limited accessible state information, errors in data entry, or mistakes made by Gemini that were not caught by OPLR's manual verification. Potential bias or measurement error introduced by these limitations likely did not substantially alter any information, as the Gemini-assisted review was utilized to find patterns rather than report exact numbers.

2. Regulatory Model Assessment and Recommendations

2.1 Menu of Regulatory Models and Factors Considered in Framework

Please see **this working document**, OPLR's Occupational Regulation Framework, for a more detailed explanation of OPLR's approach to assessing occupational regulation and evaluating different regulatory models.[85]

2.2 Model Assessment of Physical Therapy

The following table summarizes OPLR's analysis of physical therapists according to factors that OPLR determined should influence the appropriate regulatory model for an occupation. Factors that OPLR considered particularly determinative in its assessment are highlighted in bold.

[85] The document is also available on OPLR's website in the "About OPLR" section, accessible here: https://oplr.utah.gov/about-oplr/

Model Assessment of Physical Therapy	
Harm Factors	
Mechanism of Harm	Exacerbate injury, falls, cause musculoskeletal injury
Severity, Permanence, and Likelihood of Harm	Severity is typically low to moderate, although it can in rare cases be severe. Likelihood is higher if untrained
Consequence of Error	**69**
Downstream Impact	**High. Improper care can necessitate potentially invasive or risky medical intervention**
Consumer & Setting Factors	
Patient Vulnerability	Moderate. Some PTs work in acute settings with highly vulnerable patients, others work with injury on relatively healthy adults
Frequency of Physical Touch	**The practice of physical therapy requires constant physical touch**
Frequency of Private Setting	Variable. Some settings are not private (i.e. outpatient clinics) while others can be very private (home health)
Information Asymmetry	High. Patients are unlikely to know whether the physical therapy they are given is safe or effective, and even if they are harmed, they may believe it is an unavoidable side-effect of treatment
Related factors	
Level of Independence	***Structural:*** **Can work in solo practice and about 30% work in private out-patient, most others work as employees** ***Clinical:*** **High. In most settings, PTs evaluate, plan treatment, and implement treatment without input from other healthcare providers. They are more likely to be on multidisciplinary teams in acute settings, but even in those instances they utilize their own judgement**
Patient Choice	Restricted in certain settings such as home health, hospitals, and rehab clinics where patients are assigned a PT. In out-patient clinics the patient has greater choice in provider
Information Availability	Moderate. In settings where a PT is an employee of a large institution, there is not likely available information regarding their qualifications and background. In out-patient settings there may be more information about each provider

Level of Oversight	***Employers:*** **Moderate; they may have high oversight in some settings (acute hospital) and no oversight in other settings (solo practice).** *Government:* Besides licensing, the only government oversight is through CMS regulations and DHHS facility licensing, each of which may require providers meet certain criteria *Private Bodies:* Low. Insurers may provide minimal oversight by setting premiums, but there is no enforcement authority.

* O*Net Consequence of Error Ranking

2.3 Physical Therapy Role in Healthcare Landscape

PTs provide important medical care that improves physical function, mobility, and quality of life,[86,87,88,89] enabling patients to regain or retain their independence after sometimes devastating illness or injury. In certain cases, physical therapy may be substituted for more risky, expensive, and invasive medical treatments, such as opioids or surgery.[90,91,92] PTs can also have a positive effect on the healthcare system as a whole. Research suggests that early and guideline-adherent physical therapy is associated with lower costs,[93,94,95] fewer physician visits,[96] decreased frequency of imaging,[97] and less absenteeism from work.[98]

[86] Fritz et. al. (2021) Physical Therapy Referral from Primary Care for Acute back Pain with Sciatica
[87] Şahin (2017) Effectiveness of Physical Therapy and Exercise On Pain and Functional Status in Patients with Chronic Low Back Pain: a Randomized-Controlled Trial
[88] Hugues et. al. (2019) Limited evidence of physical therapy on balance after stroke: A systematic review and meta-analysis
[89] Wallace et. al. (2019) Pelvic floor physical therapy in the treatment of pelvic floor dysfunction in women
[90] Gellhorn et. a. (2012) Management Patterns in Acute Low Back Pain: The Role of Physical Therapy
[91] Frobell et. al. (2020) A Randomized Trial of Treatment for Acute Anterior Cruciate Ligament Tear
[92] Sun et. al. (2018) Association of Early Physical Therapy With Long-term Opioid Use Among Opioid-Naive Patients With Musculoskeletal Pain
[93] Fritz et. al. (2017) Cost-Effectiveness of Primary Care Management With or Without Early Physical Therapy for Acute Low Back Pain
[94] Fritz et. al. (2012) Primary Care Referral of Patients With Low Back Pain to Physical Therapy
[95] Childs et. al. (2015) Implications of early and guideline adherent physical therapy for low back pain on utilization and costs
[96] Gellhorn et. a. (2012)
[97] Fritz et. al. (2012) Primary Care Referral of Patients With Low Back Pain to Physical Therapy Impact on Future Health Care Utilization and Costs
[98] Fernández-de-las-Peñas et. al. (2019) Cost-Effectiveness Evaluation of Manual Physical Therapy Versus Surgery for Carpal Tunnel Syndrome: Evidence From a Randomized Clinical Trial

Regulatory Model Adjustments & Recommendations

3.1 Possible Adjustments

Please see **this working document**, OPLR's Occupational Regulation Framework, for a more detailed explanation of how OPLR approaches whether adjustments should be made within a recommended regulatory model.[99]

3.2 DOPL Complaint Data

The Division of Professional Licensing (DOPL) receives complaints from individuals, other state agencies, co-workers, professional associations, and licensing boards. DOPL is required to "investigate unlicensed practice in regulated professions, acts or practices inconsistent with recognized standards of conduct, allegations of gross negligence or incompetence, and patterns of gross negligence or incompetence".[100] Violations that meet the criteria for investigation are then prioritized and assigned to an investigator. DOPL may resolve investigations in a variety of ways, including: closing an investigation due to a lack of evidence; referring the case to another agency or to law enforcement if appropriate; carrying out informal or formal administrative sanctions or stipulated agreements; issuing a citation; or denying, suspending, or revoking an individual's license.

To analyze complaints sent to DOPL, OPLR used My License Office (MLO) to access closed complaints investigated by DOPL between 2017-2022 for all licenses/professions. This data contains information on the license name, the complaint type, and the disposition of the complaint, among many other data fields not relevant to OPLR's analysis. DOPL personnel helped code the complaint dispositions as either substantiated, unsubstantiated, or no jurisdiction. Substantiated complaints are those where a disposition includes some type of disciplinary action, whether formal or informal (e.g., letter of concern, verbal warning, surrender of license). Unsubstantiated complaints have dispositions without a disciplinary action (e.g., dismissed, lack of evidence, unfounded). 'No jurisdiction' complaints are complaints that may or may not have basis, but DOPL was not able to take action on the case.

OPLR filtered complaints to exclude any likely duplicates and then used substantiated complaints to calculate the number of complaints per license type or profession. OPLR estimated the complaint rate for each license type by dividing the number of substantiated complaints by the number of unique individuals who held that license type over the same period.

Complaint Case Notes Analysis

A more detailed analysis of historical case notes was conducted on a sample of physical therapy complaints closed between 2017-2022. A rough stratified random sample was pulled

[99] The document is also available on OPLR's website in the "About OPLR" section, accessible here: https://oplr.utah.gov/about-oplr/

[100] DOPL, An Explanation of the Complaint Handling Process for the Division of Occupational and Professional Licensing

from all complaints categorized as substantiated or no jurisdiction,[101] with stratification based on complaint type. All complaints related to sexual misconduct were included and complaint types more likely to include client harm (i.e. unethical conduct, incompetence/negligence) were oversampled. OPLR analysts read through case notes from twenty-one complaints and for each complaint summarized the issue, noted whether or not client harm occurred or potentially occurred, and if harm was present, the type and severity.

Limitations

There are significant limitations to this analysis, and the information collected should not be interpreted as a precise estimate of harm caused by physical therapists. DOPL data likely underestimates true harm, as many instances of harm may be handled in other ways (e.g., directly by employers), reported to other entities, or may never be reported. Additionally, some unsubstantiated complaints may have resulted in harm but the necessary evidence was not produced.

There could also be latent factors correlated with both the likelihood of complaint and the profession, systematically biasing the comparisons across professions. This is especially true in healthcare, as certain professions, by their nature, include a greater potential for harm and may generate more complaints. For example, surgeons have a higher likelihood of causing severe harm to a patient than physical therapists because surgery is inherently far riskier, not because surgeons are “less safe” or less competent than physical therapists.

For these reasons, OPLR uses DOPL complaint data as directionally informative, but avoids direct comparisons across professions wherever possible. Fine comparisons across professions are unwarranted and unsupported by these data.

3.3 DOPL Licensee Data

OPLR used DOPL licensee data queried in January of 2025 to conduct analyses on the number of licensees per year, inflow and outflow of licensees, overlap of licenses, and time with license. The dataset included individuals first licensed after 1970 to those actively licensed as of January 2025. Each row in this dataset was a unique combination of individual and license type and contained information regarding when the license was issued, the status of the license, the date the status was last updated, and the sex and year of birth of the individual. OPLR estimated the number of licensees in each year by summing the number of unique individuals whose licenses were active during any point in each year. Additionally, OPLR excluded any individual with a null or incorrect value for their license issue date and license expiration date, as OPLR could not determine how long or for what years they were actively licensed. License counts may slightly underestimate the true number of licensees due to this, but the effect is fairly negligible given

[101] No jurisdiction complaints were included in the case note analysis and not the complaint rate analysis because they are not complaints where DOPL took an action, but they may include legitimate client harm that DOPL had to refer to another agency. Although they can not be classified as “substantiated”, OPLR felt these complaints could help contextualize client harm resulting from PTs and PTAs.

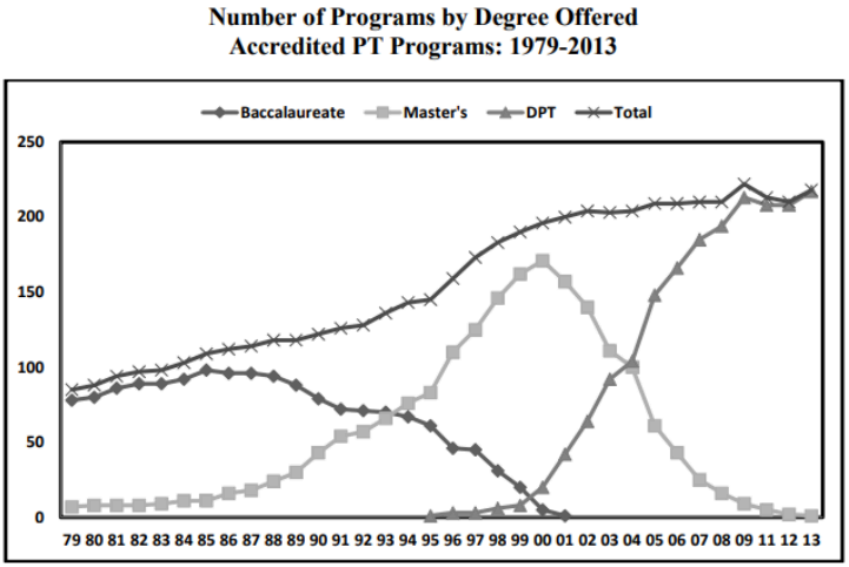
OPLR's use of the data to determine trends over time rather than estimate with precision for specific dates.

Between 2014-2024, the number of PT licenses in Utah grew with a Compound Annual Growth Rate (CAGR) of 4.6%, and the number of PTA licenses increased with a CAGR of 5.9%, both of which far outpace the growth in the population of Utah during that period (1.8%).

3.4 Physical Therapy Education Evolution

Education Evolution		
License Type	Education Change	Year
Physical Therapist (PT)[102][103]	New York University (NYU) creates the first degree program for physical therapy, **a four-year bachelor's of science**	1927
	NYU creates the first **postprofessional PhD**	1973
	Creighton University graduates the first class of **Doctor of Physical Therapy (DPT)** students	1996
	The APTA **sets a goal of requiring DPT** for professional entry by 2020	2000
	A **master's degree becomes the minimum** formal education requirement for entry level PTs	2002
	A **DPT becomes** the minimum formal education requirement for entry level PTs; **mandated by APTA/CAPTE accreditation**	2016

The graph below, published by CAPTE, illustrates how the DPT took over both Baccalaureate and Master's physical therapy programs far before it was officially required in 2016. This is likely due to the APTA's statement in 2000 calling for the DPT to become the entry level degree.



[102] Dixon, PT Education Through Time
[103] APTA, 100 Milestones of Physical Therapy

4. Rule Review

4.1 Incorrect References

Rule	Incorrect Reference	Correct Reference
R156-24b-101(3)	R156-1-107	Unsure. Reference doesn't exist
R156-24b-102(6)	58-24b-102(10)	58-24b-102(11)
R156-24b-302(5)(b)	58-24b-302(2)(d)	Unsure. It doesn't align with context of reference in rule.
R156-24b-505(1)	58-24b-505(1)	58-24b-306(1)
R156-24b-505(2)	58-24b-505(1)(e)	58-24b-306(1)(e)
R156-24b-505(2)	58-24b-502(2)(b)	58-24b-306(2)(b)
R156-24b-505(3)	58-24b-505(1)(c)	58-24b-306(1)(c)
R156-24b-505(4)	58-24b-505(1)(a)	58-24b-306(1)(a)
R156-24b-505(4)	58-24b-505(1)(e)	58-24b-306(1)(e)

5. Stakeholder Outreach

5.1 OPLR Interview Series

OPLR relied heavily on stakeholder engagement and qualitative interview data, combined with OPLR's other analysis, to conduct this review and develop recommendations. OPLR engaged with PTs, PT business owners and employers, physical therapy educators, industry associations, Utah legislators, and Utah and other state regulators. OPLR prioritized diversity of perspective and relevance to the industry in selecting stakeholders. Variety in practice setting was also prioritized, and OPLR contacted stakeholders associated with all the following settings: acute in-patient, private outpatient, hospital system outpatient, home health, and skilled nursing facilities.

Interviews were conducted in person, over the phone, and via video conferencing using semi-structured interview methods; they were conducted one-on-one and with multiple members. Extensive notes were taken for all interviews..

OPLR conducted initial interviews to understand the physical therapy industry, determine the largest issues related to safety and access, and identify potential areas for change. OPLR engaged with stakeholders later in its review to test initial findings from analysis and preliminary recommendations. OPLR reflected on and synthesized feedback across multiple discussion sessions to develop clear and achievable evidence-based recommendations.

Limitations

This interview sample was not randomly selected and, therefore, is not completely representative. OPLR spoke to individuals most likely to represent the broad aims and concerns of their groups. Additionally, OPLR did not contact "consumers" of physical therapy (i.e. physical therapy patients), so their perspectives were not incorporated into this review. Thus, the stakeholder engagement and findings from these interviews should not be understood to be fully representative of the views of all Utahns, physical therapists, physical therapy employers or clinic owners, or any other person, group, or population.

Note that stakeholders' views are not always reflected in OPLR's recommendations. OPLR is directed by Utah Code 13-1b-302 to apply specific review criteria. These can and do lead to recommendations that diverge from stakeholder preferences. A stakeholder's appearance here is not an endorsement of OPLR's recommendations as such.

5.2 Stakeholder Engagement Summary

Stakeholder Engagement Summary - Physical Therapy	
Government Stakeholders	
Utah Department of Commerce	**Margaret Busse**, Executive Director **Carolyn Dennis**, Deputy Director **Jacob Hart**, Deputy Director **Mark Steinagel**, Director, Division of Professional Licensing **Lisa Martin**, Bureau Manager, Division of Professional Licensing
Division of Professional Licensing (DOPL) Board	**Jeff Lau,** Vice Board Chair, Physical Therapies Board
Industry Stakeholders	
Industry Associations	**Katie Mansell,** Executive Director, APTA Utah **Justin Carrier**, President, APTA Utah **Drew Wilcox**, Continuing Education Liaison, APTA Utah **Chad Tenney**, Legislative Liaison, APTA Utah **Howard Quackenbush**, Chief Delegate, APTA Utah **Sarah Wallace**, Vice President, APTA Utah **Mike Sonntag**, Lobbyist, APTA Utah **Matt Hansen**, Executive Director, Homecare and Hospice Association of Utah **Allison Spangler**, President & Chief Executive Officer, Utah Health Care Association
Employers	**Joseph Kamerath**, Senior Medical Director for Rehabilitation Services, Intermountain Health **Stephen Hunter**, Director of Internal Process Control for Rehabilitation Services, Intermountain Health **Steve Crandall**, Co-Owner and Director, Hand and Orthopedic Physical Therapy Specialists **Rick Lybbert**, Chief Executive Officer, Mountain Land Physical Therapy **Clay Watson**, Owner and Physical Therapist, Western Summit Rehab
Subject-Matter Experts	
Academics, Researchers, & Clinicians	**Randy Carson**, Inpatient Medical Rehabilitation Therapy Coordinator, Neilson Rehabilitation Hospital, and Center Coordinator of Clinical Education for Acute care and Inpatient rehabilitation, University of Utah **Joel Tenbrink**, Associate Department Chair, Department of Physical Therapy, Rocky Mountain University **Pam Dibblee**, Internal Process Control Coordinator and Physical Therapist, Intermountain Health **Brad Powell**, Founder and Chief Executive Officer, Foundation Physical Therapy

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